

**Physical Therapy
PT Evaluation & Plan of Treatment**

Provider: PH OPS OF ARLINGTON
NPI: 1548907314

Certification Period: 5/14/2025 - 6/12/2025
Physical Therapy

Identification Information

Patient: WHITLOCK, DORIS J **DOB:** 5/10/1941 **Start of Care:** 5/14/2025
Payer: Wellmed (MGA)
MRN: 7746

Diagnoses

Type	Code	Description	Onset
Med	G93.41	Metabolic encephalopathy	5/9/2025
Tx	M62.59	Muscle wasting and atrophy, not elsewhere classified, multiple sites	5/9/2025

Plan of Treatment

Treatment Approaches May Include

- Therapeutic exercises (97110)
- Neuromuscular reeducation (97112)
- Gait training therapy (97116)
- Group therapeutic procedure (97150)
- Physical therapy evaluation: moderate complexity (97162)
- Therapeutic activities (97530)
- Wheelchair management training (97542)

Frequency: 6 time(s)/week

Duration: 30 day(s)

Intensity: Daily

Cert. Period: 5/14/2025 - 6/12/2025

Objective Progress / Short-Term Goals

STG #1.0 - New Goal

Pt will improve bed mobility to Ind with good safety awareness (Target: 5/27/2025)

PLOF (prior to onset)	Baseline (5/14/2025)
Ind	Min A

STG #2.0 - New Goal

Pt will improve transfers to Ind with proper hand placement for safety (Target: 5/27/2025)

PLOF (prior to onset)	Baseline (5/14/2025)
Ind	Mod A

STG #3.0 - New Goal

Pt will improve dynamic standing balance to F+ (Target: 5/27/2025)

PLOF (prior to onset)	Baseline (5/14/2025)
F+	P

Objective Progress / Long-Term Goals

LTG #1.0 - New Goal

Pt will ambulate 150ft WW MI on level surfaces with atleast 2 turns and good safety awareness.
(Target: 6/12/2025)

PLOF (prior to onset)	Baseline (5/14/2025)
150ft WW MI	25ft WW Mod A

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Payer: Wellmed (MGA)
MRN: 7746

Objective Progress / Long-Term Goals

LTG #2.0 - New Goal

Patient will improve ability to pick up objects from a standing position with Independence in order to increase independence with ADL / IADL tasks. (Target: 6/12/2025)

	PLOF (prior to onset)	Baseline (5/14/2025)
Picking up object	I	Dependent

Patient Goals: Wants to be Independent for functional mobility

Potential for Achieving Rehab Goals: Good

Focus of Plan of Treatment = Restoration

Participation = Patient/Caregiver participated in establishing POT

Original Signature: _____ Electronically signed by Deepika Meka, PT 5/15/2025 03:30:31 PM EDT
Date

I certify the need for these medically necessary services furnished under this plan of treatment while under my care from 5/14/2025 through 6/12/2025.

☐ Physician Signature Not Required

Physician Signature: _____ Electronically signed by Soroush Behnam, MD
Soroush Behnam NPI: 1710908744 Date: 5/19/2025 11:08 AM EDT

Initial Assessment / Current Level of Function & Underlying Impairments

Patient Referral and History

Current Referral	Reason for Referral / Current Illness: 84 y/o female who went to hospital with AMS and poor PO intake. Patient was found on floor at ILF with hypoxia, O2 saturation 88% on room air. At hospital, patient treated for hypokalemia, rhabdomyolysis, and recommended for dementia eval outpatient.
Oxygen	Is Oxygen needed? = No
Weight Bearing	LE Weight Bearing Status = Weight Bearing as Tolerated
Medical Hx	Prior Medical History: cervical CA, HTN, left shoulder fracture, ER visit in December 2024 after a fall
Medical Factors	Precautions: Fall risk, memory/safety deficits Contraindications = No contraindications present.
Medications	Medications Impacting Condition/Treatment: verapamil
Labs	Abnormal lab values that may impact Treatment? = No
Prior Therapy	Did Patient Receive Therapy Previously? = No
Prior Living	Prior Living Environment = Patient resided in an ILF. Prior Living Description: Patient lives in ILF and was mod I with ADL and mobility using 4WW. Current Assistance / Support = AM assistance/caregiver available, PM assistance/caregiver available
Prior Equipment	Equipment Prior to Onset: 4WW, RW, shower chair
Prior Assistance	Prior Cognitive Assistance = Daily SUP (may be left alone 6-8 hrs or more)

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Initial Assessment / Current Level of Function & Underlying Impairments

Patient Referral and History

Transition/DC Transition/Discharge Plan = Patient to return to residence at ILF.

Prior Level(s) of Function Prior Device Use = Walker

Indoor Mobility (Ambulation) = Independent

Stairs = Not Applicable

Functional Cognition = Independent

Self-Care = Independent

PLOF: Roll left and right = Independent; Sit to lying = Independent; Lying to sitting on side of bed = Independent; Sit to stand = Independent; Chair/bed-to-chair transfer = Independent; Toilet transfer = Independent; Car transfer = Independent; Walk 10 feet = Independent; Walk 50 feet with Two Turns = Independent; Walk 150 feet = Independent; Walking 10 feet on uneven surfaces = Independent; 1 step (curb) = Not applicable; Wheel 50 feet with two turns = Not applicable; Wheel 150 feet = Not applicable; Picking up object = Independent

Fall Risk Assessment

History of Falls Has Patient fallen in past year? = Yes; How many times? = Exact number unknown; Was patient injured from fall? = Unknown

Steadiness Does Patient feel unsteady when standing? = No; Does Patient feel unsteady when walking? = No

Fear of Falling Does Patient worry about falling? = No

Functional Mobility Assessment

Bed Mobility Roll left and right = Partial/moderate assistance

Sit to lying = Partial/moderate assistance

Lying to sitting on side of bed = Partial/moderate assistance

Transfers Sit to stand = Partial/moderate assistance

Chair/bed-to-chair transfer = Partial/moderate assistance

Toilet transfer = Partial/moderate assistance

Car transfer = Not attempted due to medical conditions or safety concerns

Ambulation Walk 10 feet = Partial/moderate assistance

Walk 50 feet with Two Turns = Not attempted due to medical conditions or safety concerns

Walk 150 feet = Not attempted due to medical conditions or safety concerns

Walking 10 feet on uneven surfaces = Not attempted due to medical conditions or safety concerns

Gait Pattern / Deviations: Patient exhibits decreased stride length, decreased velocity and discontinuous steps associated with forward lean of trunk and underlying causes that might be modifiable include: muscle paresis/weakness.

Curbs/Stairs 1 step (curb) = Not applicable

W/C Mobility Resident uses a wheelchair and/or scooter? = Yes

Wheel 50 feet with two turns = Partial/moderate assistance

Type of wheelchair or scooter used? = Manual

Wheel 150 feet = Not attempted due to medical conditions or safety concerns

Type of wheelchair or scooter used? = Manual

Other Picking up object = Dependent

Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 4

Mobility Performance Raw Score = 27

Musculoskeletal System Assessment

LE ROM RLE ROM = WFL; LLE ROM = WFL

RLE Strength RLE Strength = WFL

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Payer: Wellmed (MGA)
MRN: 7746

Initial Assessment / Current Level of Function & Underlying Impairments

Musculoskeletal System Assessment

LLE Strength LLE Strength = WFL
Contracture Functional Limitations Present due to Contracture = No

Other System/Condition Assessment

Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No
Cardiovascular CardioPulmonary System = Functional for age and condition
Pain Patient has pain that interferes/limits functional activity? = No; Patient has pain that interferes with sleep? = No; Pain Assessment Method = Patient verbalized pain level.

Objective Tests and Measures

Test/Sit Balance Sitting Balance Scale = NT

Assessment Summary

Communication Ability to Express Ideas and Wants = Understood; Ability to Understand Others = Understands; Follows 1-Step Directions = Independently with prompts/cues
Cognition Decision Making Ability for Routine Activities = Moderately Impaired
Reason for Therapy Clinical Impressions/Reason for Skilled Services: Pt would benefit from Continued skilled PT treatment is medically necessary to increase strength, balance, activity tolerance and safety awareness in order to improve ADL and functional mobility independence. Without skilled PT intervention, patient at risk for falls, injury and further caregiver dependence.
Complexities Barriers Likely to Impact Discharge to Next Level = Multiple medications/management required
Patient Characteristics that may Impact Treatment = Multiple medical conditions/history, Multiple medications
Intervention Modes Can interventions be provided using treatment modes other than individual? = Yes; Will multiple modes provide a value-added benefit? = Yes; Will multiple modes enhance the therapeutic experience? = Yes; Is patient agreeable to modes other than individual? = Yes; Modes recommended = Group, Concurrent; Maximum number of patients for any group = Select All; Justification for group recommendation: Patient will benefit from group therapy as an adjunct to individual therapy to facilitate learning that's provided in peer groups and to facilitate patient's ability to both receive and give feedback to support learning new skills.; Justification for concurrent recommendation: Patient will benefit from concurrent therapy as an adjunct to individual therapy to facilitate learning that's provided in peer therapeutic modes that would not occur in individual therapy and to facilitate patient's opportunity to dynamically receive and give feedback from peers to support health literacy.

Exercise Prescription

Purpose Purpose of Exercise = Strength
Method Method to Prescribe = Patient's Perceived Resistance
Resistance Resistance for Strength at Somewhat Hard to Hard Level = 3 pounds
Strength Muscle Groups@ 1 set, 15 - 20 reps, for Strength: BLE Muscle Groups

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MRN: 7746		

**Physical Therapy
PT Therapy Progress Report**

Provider: PH OPS OF ARLINGTON
NPI: 1548907314

Dates of Service: 5/14/2025 - 5/21/2025
Physical Therapy

Identification Information

Patient: WHITLOCK, DORIS J **DOB:** 5/10/1941 **Start of Care:** 5/14/2025
Payer: Wellmed (MGA)
MRN: 7746

Diagnoses

Type	Code	Description	Onset
Med	G93.41	Metabolic encephalopathy	5/9/2025
Tx	M62.59	Muscle wasting and atrophy, not elsewhere classified, multiple sites	5/9/2025

Patient was seen for 5 day(s) during the 5/14/2025 - 5/21/2025 progress period.

Objective Progress / Short-Term Goals

STG #1.0 - Continue

Pt will improve bed mobility to Ind with good safety awareness

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/14/2025)	Current (5/21/2025)
Ind	Min A	Min A	SUP
Comments: SUP			

STG #2.0 - Continue

Pt will improve transfers to Ind with proper hand placement for safety

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/14/2025)	Current (5/21/2025)
Ind	Mod A	Mod A	SUP
Comments: SUP			

STG #3.0 - Continue

Pt will improve dynamic standing balance to F+

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/14/2025)	Current (5/21/2025)
F+	P	P	P+
Comments: P+			

Objective Progress / Long-Term Goals

LTG #1.0 - Continue

Pt will ambulate 150ft WW MI on level surfaces with atleast 2 turns and good safety awareness.

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/14/2025)	Current (5/21/2025)
150ft WW MI	25ft WW Mod A	25ft WW Mod A	150 ft , RW, SUP
Comments: 150 ft , RW, SUP			

LTG #2.0 - Continue

Patient will improve ability to pick up objects from a standing position with Independence in order to increase independence with ADL / IADL tasks.

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/14/2025)	Current (5/21/2025)
Picking up object	I	Dependent	Not applicable
Comments:			

Precautions / Prior Living / Discharge / Transition Plan

Medications Medications Impacting Condition/Treatment: verapamil
Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Weight Bearing LE Weight Bearing Status = Weight Bearing as Tolerated

**Physical Therapy
PT Therapy Progress Report**

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MRN: 7746

Precautions / Prior Living / Discharge / Transition Plan

Medical Hx Prior Medical History: cervical CA, HTN, left shoulder fracture, ER visit in December 2024 after a fall
Prior Living Prior Living Environment = Patient resided in an ILF.
 Prior Living Description: Patient lives in ILF and was mod I with ADL and mobility using 4WW.
Prior Assistance Prior Cognitive Assistance = Daily SUP (may be left alone 6-8 hrs or more)
Prior Equipment Equipment Prior to Onset: 4WW, RW, shower chair
Discharge Plan Transition/Discharge Plan = Patient to return to residence at ILF.
 Assistance/Support to be Provided = AM assistance/caregiver available
 Barriers Likely to Impact Discharge to Next Level = Multiple medications/management required
 Likely need for caregiver assistance in next setting for = Medication management, Supervision and safety
 Patient Characteristics that may Impact Treatment = Multiple medical conditions/history, Multiple medications

Functional Skills Assessment

Bed Mobility Roll left and right = Supervision or touching assistance
 Sit to lying = Supervision or touching assistance
 Lying to sitting on side of bed = Supervision or touching assistance
Transfers Sit to stand = Supervision or touching assistance
 Chair/bed-to-chair transfer = Supervision or touching assistance
 Toilet transfer = Supervision or touching assistance
 Car transfer = Not applicable
Ambulation Walk 10 feet = Supervision or touching assistance
 Walk 50 feet with Two Turns = Supervision or touching assistance
 Walk 150 feet = Supervision or touching assistance
 Walking 10 feet on uneven surfaces = Supervision or touching assistance
Curbs/Stairs 1 step (curb) = Supervision or touching assistance
 4 steps = Not applicable
W/C Mobility Resident uses a wheelchair and/or scooter? = No
Other Picking up object = Not applicable
Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 9
 Mobility Performance Raw Score = 47

Assessment and Summary of Skilled Services

Interventions Skilled Interventions Provided: Direct, hands-on care with patient this reporting period focused on the following skilled interventions: Gait training, OCK exercise, standing balance training, transfer training, sit<> stand training.
Patient Progress Progress & Response to Treatment: Patient exhibits ability to participate in tasks of increased complexity to further enhance functional independence.
Balance Patient sits unsupported x 30 seconds with feet flat on floor and no back support? = Yes; Patient stands without UE support w/AD as needed x 10 seconds? = No
Target Heart Rate Target Heart Rate Range: 82 to 95 BPM
Test/Sit Balance Sitting Balance Scale = NT
Purpose Purpose of Exercise = Strength
Method Method to Prescribe = Patient's Perceived Resistance
Resistance Resistance for Strength at Somewhat Hard to Hard Level = 3 pounds
Strength Muscle Groups@ 1 set, 15 - 20 reps, for Strength: BLE Muscle Groups

**Physical Therapy
PT Therapy Progress Report**

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Dates of Service: 5/14/2025 - 5/21/2025
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MRN: 7746

Assessment and Summary of Skilled Services

Supervision PT/Assistant Supervision: Communication regarding the patient's objective progress and treatment progression has occurred with the treating clinician(s) this reporting period.

Communication Team Communication/Collaboration: Care planning conference held with interdisciplinary team this PR period - functional skills and discharge plans reviewed.

Justification for Continued Skilled Services

Impairments Remaining Impairments: balance deficits and decreased dynamic balance.

Continued Skill Reason for Skilled Services: Continued PT services are necessary in order to enhance rehab potential, facilitate discharge planning and increase independence with gait in order to enhance patient's quality of life by an improved ability to decrease level of assistance from caregivers and safely return to community . Due to the documented physical impairments and associated functional deficits, without skilled therapeutic intervention, the patient is at risk for: falls.

Intervention Modes Can interventions be provided using treatment modes other than individual? = Yes; Will multiple modes provide a value-added benefit? = Yes; Will multiple modes enhance the therapeutic experience? = Yes; Is patient agreeable to modes other than individual? = Yes; Modes recommended = Group, Concurrent; Maximum number of patients for any group = Select All; Justification for group recommendation: Patient will benefit from group therapy as an adjunct to individual therapy to facilitate learning that's provided in peer groups and to facilitate patient's ability to both receive and give feedback to support learning new skills.; Justification for concurrent recommendation: Patient will benefit from concurrent therapy as an adjunct to individual therapy to facilitate learning that's provided in peer therapeutic modes that would not occur in individual therapy and to facilitate patient's opportunity to dynamically receive and give feedback from peers to support health literacy.

Plan of Tx Focus Focus of Plan of Treatment = Restoration

Original Signature: _____ Electronically signed by Manuel Sanchez, PTA 6/5/2025 10:48:04 AM CDT
Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 6/5/2025 10:59:47 AM CDT
Date

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF ARLINGTON

WHITLOCK, DORIS J

Identification Information

Patient: WHITLOCK, DORIS J

MRN: 7746

DOB: 5/10/1941

Date of Service: 5/27/2025

Completed Date: 5/27/2025

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97110 97110: OCK exercise for B LE/BUE to increase circulation, ROM, strength, and activity tolerance to facilitate symmetrical reciprocation and bilateral integration; instructions for continuous reciprocation for recumbent stepper with light to moderate resistance. Supervised by Deepika Meka RPT.

97116 97116: Gait training to normalizr gait pattern; with RW, 75 ft x 3, SUP, with min verbal cues to initiate energy conservation techniques in between intervals. Supervised by Deepika Meka, RPT.

Response to Tx Response to Session Interventions: Pt able to safely transfer and ambulate in room with RW for ADLs.

Complexities Barriers impacting treatment? = No

Functional Status as a Result of Skilled Interventions

Bed Mobility Roll left and right = Setup or clean-up assistance

Sit to lying = Setup or clean-up assistance

Lying to sitting on side of bed = Setup or clean-up assistance

Transfers Sit to stand = Setup or clean-up assistance

Chair/bed-to-chair transfer = Setup or clean-up assistance

Toilet transfer = Setup or clean-up assistance

Ambulation Walk 10 feet = Supervision or touching assistance

Walk 50 feet with Two Turns = Supervision or touching assistance

Walk 150 feet = Supervision or touching assistance

Walking 10 feet on uneven surfaces = Supervision or touching assistance

Curbs/Stairs 1 step (curb) = Supervision or touching assistance

4 steps = Supervision or touching assistance

W/C Mobility Resident uses a wheelchair and/or scooter? = No

Original Signature: _____ Electronically signed by Manuel Sanchez, PTA 5/27/2025 02:32:02 PM CDT
Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 5/27/2025 10:27:42 PM CDT
Date

Date of Service: 5/26/2025

Completed Date: 5/26/2025

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97110 97110: Patient provided with CKC exercises L1 15 minutes to improve with B LE closed chain ROM and functional activity tolerance.

97116 97116: Patient provided with treatment session initiating with gait training with minimal verbal cues to maintain heel to toe walking pattern and improve stride and step length with activation of B hip flexors and B ankle DF for foot clearance. Pt able to improve with gait distance and ambulated ~150 feet with RW support.

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF ARLINGTON

WHITLOCK, DORIS J

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Vishal Narendra Bhavnagari; PT 5/26/2025 09:53:00 PM CDT
Date

Date of Service: 5/25/2025 **Completed Date: 5/25/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97110 97110: Patient provided with CKC exercises L1 15 minutes to improve with B LE closed chain ROM and functional activity tolerance.
97116 97116: Patient provided with treatment session initiating with gait training with minimal verbal cues to maintain heel to toe walking pattern and improve stride and step length with activation of B hip flexors and B ankle DF for foot clearance. Pt able to improve with gait distance and ambulated ~150 feet with RW support.
Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Vishal Narendra Bhavnagari; PT 5/25/2025 10:51:03 PM CDT
Date

Date of Service: 5/20/2025 **Completed Date: 5/20/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97116 97116: Gait training to normalize gait pattern; with RW with instructions for step-through pattern and scanning ahead, with min verbal cues to initiate energy conservation techniques, 75 ft x 3, SUP.
97530 97530: Education and discussion about status and progression with LTGs/STGs with patient's family and staff to facilitate proper and safe discharge plan.
97150 97150: Group therapy to improve, ROM, strength, somatic proprioceptive receptors, standing activity toleranc with proper balance with proper BOS/COM, increase socialization, increase morale, and qaltiy of life to facilitate functional task performance with ADLs; instruction for introduction for warm-ups and activities to energize/stimulate with simultaneous social interaction with others using cognitive imagery cues, inspirational dialogue and music stimuli to increase endorphines in order to reduce risk for depression, reduce risk for social isolation and enhance rehab potential; instructions for OCK/CCK exercise in seated and in standing position with // bars for UE support, at 3 x 10-20 for B LE mms with gravity resistance. Education on proper precautions. Also instructions for PLB between tasks and sets to increase aeration and proper exchange of proper gases during SOB. Also instructions for pressure relief in seated position with wt shifting techniques. Group peromed with total of 6 patients. Supervised by Deepika Meka,RPT.
Response to Tx Response to Session Interventions: Pt able to ambulate in room with RW to facilitate ADLs.
Complexities Barriers impacting treatment? = No

Functional Status as a Result of Skilled Interventions

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF ARLINGTON

WHITLOCK, DORIS J

Bed Mobility Roll left and right = Supervision or touching assistance
 Sit to lying = Supervision or touching assistance
 Lying to sitting on side of bed = Supervision or touching assistance

Transfers Sit to stand = Supervision or touching assistance
 Chair/bed-to-chair transfer = Supervision or touching assistance
 Toilet transfer = Supervision or touching assistance

Ambulation Walk 10 feet = Supervision or touching assistance
 Walk 50 feet with Two Turns = Supervision or touching assistance
 Walk 150 feet = Supervision or touching assistance

Original Signature: _____ Electronically signed by Manuel Sanchez, PTA 5/20/2025 02:43:50 PM CDT
 Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 5/20/2025 11:11:35 PM CDT
 Date

Revision Signature: _____ Electronically signed by Manuel Sanchez, PTA 5/20/2025 02:56:34 PM CDT
 Date

[DORIS WHITLOCK] : Treatment Encounter Note(s)
Discipline: PT

Changes For: 5/20/2025 2:56:34 PM	
Assessment	
Changes	Modified By
Assessment added : 97530 Therapeutic Activities	Sanchez, Manuel

Date of Service: 5/19/2025 **Completed Date: 5/20/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits

Contraindications Contraindications = No contraindications present.

Pre-Tx Is skilled therapy needed to address pain? = Nursing to address

97110 97110: Pt utilized nustep x 15 minutes level 4 to increase strength and endurance. Pt was instructed in B LE ther ex in most planes of motion while standing w/ 2WW

97116 97116: Pt completed GT 120' SBA 2WW

Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Amanda Whitmore PTA 5/22/2025 08:47:59 AM CDT
 Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 5/22/2025 09:51:24 AM CDT
 Date

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF ARLINGTON

WHITLOCK, DORIS J

Date of Service: 5/16/2025 **Completed Date: 5/19/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97116 97116: Pt completed GT 120' SPV 2WW
97150 97150: Patient participated in cognition group, energy conservation group, functional activities group and therapeutic exercise group ; goal of group was to successfully progress patient with individual treatment plan/goals by addressing patient's strength impairments, gross motor coordination deficits and decreased functional capacity ; skilled service provision included B LE & UE ther ex, sing a longs for socialization and cognition, sequencing, deep breathing techniques ; 4 patients participated in the group.
Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Amanda Whitmore PTA 5/19/2025 06:27:50 AM CDT
Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 5/19/2025 10:40:00 PM CDT
Date

Date of Service: 5/15/2025 **Completed Date: 5/15/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
97530 97530: Skilled interventions focused on bed mobility training to increase functional skills, instruction in use of handrails to facilitate rolling to side(s) to come to sitting, training in rolling, scooting, bridging to facilitate independence in bed mobility, training in supine/side-lying to/from sitting edge of bed and training in safe sit to stand/stand to sit mobility. Pt ambulated to/from the bathroom using a RW with Min/CGA.
Response to Tx Response to Session Interventions: actively participates with skilled interventions.

Original Signature: _____ Electronically signed by Gladys Nyachwaya, PTA 5/15/2025 04:14:36 PM CDT
Date

Cosignature: _____ Electronically co-signed by Deepika Meka, PT 5/15/2025 10:54:41 PM CDT
Date

Date of Service: 5/14/2025 **Completed Date: 5/15/2025**

Summary of Daily Skilled Services

Precautions Precautions: Fall risk, memory/safety deficits
Contraindications Contraindications = No contraindications present.
Pre-Tx Is skilled therapy needed to address pain? = Nursing to address
Evaluation 97162: Refer to the documented Plan of Treatment for patient history, comorbidities and/or personal factors that impact the plan, in addition to the examination findings and clinical presentation.
97530 97530: Skilled interventions focused on instruction in scooting forward, backward, side to side in sitting, instruction in scooting forward, backward, side to side in supine and training in rolling, scooting, bridging to facilitate independence in bed mobility. Transfer training to increase functional task performance.
Response to Tx Response to Session Interventions: compliant with skilled interventions.

**Physical Therapy
Treatment Encounter Note(s)**

Provider: PH OPS OF ARLINGTON

WHITLOCK, DORIS J

Original Signature:		Electronically signed by Deepika Meka, PT		5/15/2025 02:32:54 PM CDT
				Date

**Physical Therapy
PT Discharge Summary**

Provider: PH OPS OF ARLINGTON
NPI: 1548907314

Dates of Service: 5/14/2025 - 5/27/2025
Physical Therapy

Identification Information

Patient: WHITLOCK, DORIS J **DOB:** 5/10/1941
Payer: Wellmed (MGA)
MRN: 7746

D/C Destination:

D/C Reason: Discharged per Physician or Case Manager

Diagnoses

Type	Code	Description	Onset
Med	G93.41	Metabolic encephalopathy	5/9/2025
Tx	M62.59	Muscle wasting and atrophy, not elsewhere classified, multiple sites	5/9/2025

Patient was seen for 3 day(s) during the 5/22/2025 - 5/27/2025 progress period.

Objective Progress/Functional Comparison with Goals

Short-Term Goals

STG #1.0 - Discontinue on 05/27/2025

Pt will improve bed mobility to Ind with good safety awareness

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/21/2025)	Discharge (5/27/2025)
Ind	Min A	SUP	Supervision

Comments:

STG #2.0 - Discontinue on 05/27/2025

Pt will improve transfers to Ind with proper hand placement for safety

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/21/2025)	Discharge (5/27/2025)
Ind	Mod A	SUP	Supervision

Comments:

STG #3.0 - Discontinue on 05/27/2025

Pt will improve dynamic standing balance to F+

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/21/2025)	Discharge (5/27/2025)
F+	P	P+	F-

Comments:

Long-Term Goals

LTG #1.0 - Discontinue on 05/27/2025

Pt will ambulate 150ft WW MI on level surfaces with atleast 2 turns and good safety awareness.

PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/21/2025)	Discharge (5/27/2025)
150ft WW MI	25ft WW Mod A	150 ft , RW, SUP	150ft WW Supervision

Comments:

**Physical Therapy
PT Discharge Summary**

Provider: PH OPS OF ARLINGTON
NPI: 1548907314

Dates of Service: 5/14/2025 - 5/27/2025
Physical Therapy

Identification Information

Patient: WHITLOCK, DORIS J **DOB:** 5/10/1941
Payer: Wellmed (MGA)
MRN: 7746

D/C Destination:

D/C Reason: Discharged per Physician or Case Manager

LTG #2.0 - Discontinue on 05/27/2025

Patient will improve ability to pick up objects from a standing position with Independence in order to increase independence with ADL / IADL tasks.

	PLOF (prior to onset)	Baseline (5/14/2025)	Previous (5/21/2025)	Discharge (5/27/2025)
Picking up object	I	Dependent	Not applicable	Supervision or touching assistance

Comments:

Status / Prior Living / Discharge

Weight Bearing LE Weight Bearing Status = Weight Bearing as Tolerated
Medical Hx Prior Medical History: cervical CA, HTN, left shoulder fracture, ER visit in December 2024 after a fall
Prior Living Prior Living Environment = Patient resided in an ILF.
Prior Living Description: Patient lives in ILF and was mod I with ADL and mobility using 4WW.
Prior Assistance Prior Cognitive Assistance = Daily SUP (may be left alone 6-8 hrs or more)
Prior Equipment Equipment Prior to Onset: 4WW, RW, shower chair
D/C Location Discharge Location = Patient discharged to resident at ILF.
Assistance/Support to be Provided = AM assistance/caregiver available

Functional Skills Assessment

Bed Mobility Roll left and right = Supervision or touching assistance
Sit to lying = Supervision or touching assistance
Lying to sitting on side of bed = Supervision or touching assistance
Transfers Sit to stand = Supervision or touching assistance
Chair/bed-to-chair transfer = Supervision or touching assistance
Toilet transfer = Supervision or touching assistance
Car transfer = Not attempted due to medical conditions or safety concerns
Ambulation Walk 10 feet = Supervision or touching assistance
Walk 50 feet with Two Turns = Supervision or touching assistance
Walk 150 feet = Supervision or touching assistance
Walking 10 feet on uneven surfaces = Supervision or touching assistance
Stairs/Curbs 1 step (curb) = Not applicable
W/C Mobility Resident uses a wheelchair and/or scooter? = Yes
Wheel 50 feet with two turns = Supervision or touching assistance
Type of wheelchair or scooter used? = Manual
Wheel 150 feet = Supervision or touching assistance
Type of wheelchair or scooter used? = Manual
W/C Management = Not attempted due to medical conditions or safety concerns
Other Picking up object = Supervision or touching assistance
Mobility Score Mobility Function Score (ranges from 0 - 12; 12 being the highest function) = 9
Mobility Performance Raw Score = 46

**Physical Therapy
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D/C Reason: Discharged per Physician or Case Manager

Assessment and Summary of Skilled Services

Skilled Interventions Skilled Interventions Provided: killed treatment interventions included instructing and training patient in energy conservation techniques, proper body mechanics, safe transfer techniques, safety precautions and use of assistive device(s) in order to improve functional mobility status.

Patient Progress Progress & Response to Treatment: Patient has made consistent progress with skilled interventions.

Communication Team Communication/Collaboration: Participated in care planning meeting, Reviewed patient's plan of treatment and treatment services with interdisciplinary team members and Treatment results communicated to Interdisciplinary Team.

Test/Sit Balance Sitting Balance Scale = NT

Discharge Recommendations and Status

D/C Recs Discharge Recommendations: HHS, Supervision, WW

Target Heart Rate Target Heart Rate Range: 82 to 95 BPM

Restorative Programs Restorative Program Established/Trained = Not Indicated at This Time

Functional Maintenance Functional Maintenance Program Established/Trained = Not Indicated at This Time

Prognosis Prognosis to Maintain CLOF = Excellent with consistent staff support

Original Signature: _____	Electronically signed by Deepika Meka, PT	6/2/2025 11:17:31 PM CDT	Date
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